GI consultation, new patient with trouble swallowing

A full doctor-patient conversation and the finished note it should produce, laid out in the Sully GI Consultation Note template. Use it to test an AI scribe, train new scribes, or see what a complete note captures.

Patient	58-year-old man (fictional)
Visit	New patient consultation, in person
Length	About 14 minutes, 69 turns
Matching template	GI Consultation Note · sully.ai/templates/gi-consultation-note

A Visit transcript

Doctor	Hi, Mr. Alvarez? I'm Dr. Okafor, one of the GI doctors here. Come on in, have a seat.
Patient	Thanks. Raymond's fine.
Doctor	Raymond, good. So Dr. Lee sent you over, and her note says something about food getting stuck. Tell me in your own words what's been going on.
Patient	Yeah, so, um, it started maybe three, four months ago. At first it was just like, steak or chicken, I'd swallow and it felt like it sat right here. *(points to lower chest)* I'd have to drink water to push it down.
Doctor	Right there behind the breastbone, lower part.
Patient	Yeah. And now it's bread too. Last week I had a sandwich and I had to go to the bathroom and bring it back up.
Doctor	OK. Liquids, any trouble with water, coffee?
Patient	No, liquids are fine. It's just solid stuff.
Doctor	And has it been getting worse steadily, or does it come and go?
Patient	Worse. Definitely worse. I've started cutting everything really small. My wife noticed, she's like, why are you eating like a kid.
Doctor	*(laughs)* Fair. Have you lost any weight?
Patient	I think so. My belt's on a new hole. I was about two-fifteen in the summer. I weighed myself Sunday, I was two-oh-one.
Doctor	So roughly fourteen pounds in a few months, without trying.
Patient	Without trying, yeah.

Doctor	Any heartburn? Burning in the chest, sour taste coming up?
Patient	I've had heartburn for years. Like, twenty years. I take the, um, the purple pill sometimes. Not every day. When it's bad.
Doctor	Omeprazole, or the one called Nexium?
Patient	Nexium. The over-the-counter one.
Doctor	And how often would you say you take it?
Patient	Maybe two, three times a week? It used to be more. Actually the heartburn's been less lately, which is weird.
Doctor	OK, that's useful. Any pain when you swallow, like a sharp pain as food goes down, versus just the sticking?
Patient	Not really pain. Just pressure.
Doctor	Any vomiting blood, or anything that looked like coffee grounds?
Patient	No.
Doctor	Black or tarry stools?
Patient	Um. They've been darker? But I started an iron pill, so I figured that's why.
Doctor	Who started the iron?
Patient	Dr. Lee. My blood count was low on the physical.
Doctor	Yes, I see it. Hemoglobin of 10.9, and the iron studies were low. So she started ferrous sulfate in September. Before the iron, were the stools normal color?
Patient	I think so. I didn't really look, honestly.
Doctor	That's fine, most people don't. Any change in your bowel habits otherwise, diarrhea, constipation?
Patient	A little constipated since the iron.
Doctor	Common with iron. Have you ever had a colonoscopy?
Patient	Once. When I turned fifty. They took out a polyp. They told me come back in five years and, uh, I didn't.
Doctor	OK. We'll come back to that. Any trouble with your voice, coughing when you eat, food going down the wrong way?
Patient	No.
Doctor	Do you smoke?
Patient	I quit eight years ago. I smoked about a pack a day for, I don't know, thirty years.
Doctor	And alcohol?
Patient	Couple of beers on the weekend. Not much.
Doctor	Any family history of cancer, especially stomach, esophagus, colon?
Patient	My dad had colon cancer. He was in his seventies. My uncle had something in his stomach, I don't know exactly what.

Doctor	OK. Other medical problems?
Patient	Blood pressure. And my cholesterol.
Doctor	I have lisinopril 20 and atorvastatin 40. Anything else? Aspirin, ibuprofen, Aleve?
Patient	I take ibuprofen for my knee. Pretty much every day, honestly. Two in the morning.
Doctor	Two of the 200s?
Patient	Yeah.
Doctor	OK, that's important, so thank you. Any allergies to medications?
Patient	Penicillin. I got a rash as a kid.
Doctor	Alright. Let me take a look at you. *(exam)* Take a deep breath for me. Again. Good. Lie back. Any tenderness when I press here?
Patient	No.
Doctor	Here?
Patient	No, that's fine.
Doctor	OK, you can sit up. So let me tell you what I'm thinking. Food sticking that's getting worse over a few months, solids first, with weight loss and a low blood count, is something we have to look at directly, and soon. There are a few things that cause it. Long-standing reflux can cause a narrowing, a scar-type ring, which is common and very fixable. Reflux can also cause changes in the lining called Barrett's. And in someone with your history of reflux and smoking, we need to make sure there's no growth in the esophagus. I'm not saying that's what it is, but the pattern means we don't wait on it.
Patient	OK. That's, yeah. That's kind of what I was worried about.
Doctor	That's a reasonable worry, and the way to answer it is to look. I'd like to do an upper endoscopy, a camera down the throat, while you're sedated. If I find a narrowing I can often stretch it during the same procedure. And I'll take biopsies of anything that looks off. I want this in the next week or two, not months.
Patient	OK.
Doctor	And since you're overdue for your colonoscopy, you have a low blood count from low iron, and your dad had colon cancer, I'd like to do the colonoscopy the same morning. One sedation, both done.
Patient	Yeah, let's do that. Get it over with.
Doctor	Good. Until then, a few things. I'd like you to stop the ibuprofen. It can irritate the stomach and esophagus and it may be part of why your blood count is low. Tylenol is fine for the knee, up to 3,000 milligrams a day. And I want you on the Nexium every day, 40 milligrams, thirty minutes before breakfast, instead of as needed.
Patient	Every day. OK.
Doctor	Soft foods, small bites, sit up while you eat. If food gets stuck and doesn't pass, or you can't swallow your own saliva, that's an emergency room visit, not a wait-and-see.
Patient	Got it.

Doctor	And keep taking the iron for now. I'll order a repeat blood count and iron panel today so we have a fresh number before the procedure. Any questions for me?
Patient	How long's the procedure?
Doctor	Both together, about forty-five minutes, and you'll be in the building around three hours with prep and recovery. You'll need a driver. The nurse will go over the bowel prep with you on the way out.
Patient	OK. Thanks, doc.
Doctor	You're welcome. We'll get answers quickly.

B Finished note

Patient / MRN / DOB: Raymond Alvarez (fictional), 58 M

Date of service: synthetic. **Referral:** Dr. Lee (PCP), progressive dysphagia and iron deficiency anemia. New patient. In person.

History

Presenting problem: 3 to 4 months of progressive solid-food dysphagia, localized to the lower retrosternal area. Began with meat, now includes bread. One episode of regurgitation of a sandwich last week. Liquids pass normally. No odynophagia. Patient has started cutting food into small pieces.

Alarm features: Weight loss present (about 215 to 201 lb, ~14 lb, unintentional). Dysphagia present, progressive. Anemia present (Hgb 10.9 g/dL, low iron studies, ferrous sulfate started 2 months ago). Melena uncertain: darker stools since starting iron; baseline color not recalled. No hematemesis. No persistent vomiting. Family history: father with colorectal cancer in his 70s; uncle with gastric malignancy (type unknown).

GI history and medications: ~20-year history of heartburn, on OTC esomeprazole 2 to 3 times weekly, with recent decrease in heartburn. Screening colonoscopy at 50 with one polyp removed; 5-year surveillance missed. Daily ibuprofen 400 mg for knee pain. Lisinopril 20 mg, atorvastatin 40 mg, ferrous sulfate. Allergy: penicillin (childhood rash).

Family and social: Former smoker, ~30 pack-years, quit 8 years ago. Alcohol: a few beers on weekends. Family history as above.

Examination and data

Examination: Lungs clear. Abdomen soft, non-tender, no masses.

Data reviewed and interpreted: PCP labs reviewed and interpreted: Hgb 10.9 g/dL with low iron studies, consistent with iron deficiency anemia.

Assessment and plan

Assessment:

1. Progressive solid-food dysphagia with weight loss and iron deficiency anemia. Differential: peptic stricture from long-standing GERD; Barrett's esophagus; esophageal adenocarcinoma, given GERD, smoking history, age, alarm features and lessening heartburn; Schatzki ring (less likely given weight loss).
2. Iron deficiency anemia, possibly from NSAID-related mucosal injury, upper GI lesion, or colonic source. Overdue polyp surveillance and first-degree family history of CRC.

Plan:

- EGD with biopsies and dilation if indicated, within 1 to 2 weeks. Indication: progressive dysphagia with alarm features.

- Colonoscopy same session. Indication: iron deficiency anemia, overdue polyp surveillance, family history of CRC.
- Stop ibuprofen. Acetaminophen up to 3 g/day for knee pain.
- Esomeprazole 40 mg daily, 30 minutes before breakfast.
- Repeat CBC and iron panel today.
- Continue ferrous sulfate.
- Diet: soft foods, small bites, upright while eating.
- Return precautions: food impaction or inability to handle secretions, go to ED.
- Prep instructions given by nursing. Patient needs a driver.

Coding

Level of service: New patient. MDM moderate to high (undiagnosed new problem with uncertain prognosis; prescription drug management; decision for procedure with identified risk factors).

> **How to use this sample.** Run the transcript through your scribe and compare its draft with the finished note. Check the details that are easy to miss: doses, which side of the body, what the patient said no to, and every follow-up step. For 536 more transcripts, many with audio and reference notes, see github.com/aniquethaplawala/open-clinical-encounters.

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